

Question:

An 8-year-old girl is brought to the emergency department because of a 2-day history of low-grade fever, itchy rash, and generalized joint pain. The rash initially started in the antecubital and popliteal fossae and then spread to her trunk and distal extremities. One week ago, she was diagnosed with acute sinusitis and was started on amoxicillin. She has no history of adverse drug reactions and immunizations are up-to-date. Her temperature is 37.5°C (99.5°F), pulse is 90/min, and blood pressure is 110/70 mm Hg. Physical examination shows periorbital edema and multiple erythematous, annular plaques of variable sizes over her entire body. One of the lesions in the right popliteal fossa has an area of central clearing and the patient's mother reports that it has been present for over 24 hours. Urinalysis is normal. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Serum sickness-like reaction
- B. Stevens-Johnson syndrome
- C. Pemphigus vulgaris
- D. Drug reaction with eosinophilia and systemic symptoms

Reasoning + Answer:

<think> The clinical presentation of the child with a rash that started on the flexural areas and then spread to the rest of the body and the target lesions and time course of the rash and the history of a recent antibiotic use is indicative of a serum sickness like reaction which is an immune complex mediated hypersensitivity reaction that can occur after starting a new medication and shows a rash that begins in dependent areas and then spreads to the rest of the body and can have target lesions and fever and joint pain </think>
<answer> A. Serum sickness-like reaction </answer>